جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : ام السيد السيد محمد يوسف
المحافظة : القاهره **الحي / المدينة :** المطريه **العنوان :** --
رقم البطاقة : 24606151302064 **رقم التليفون :** 01151128718 **تاريخ تسجيل الإستمارة :** 18-10-2026
العمر(تاريخ الميلاد) : 15-06-1946 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: ام السيد السيد محمد يوسف
National ID: 24606151302064

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

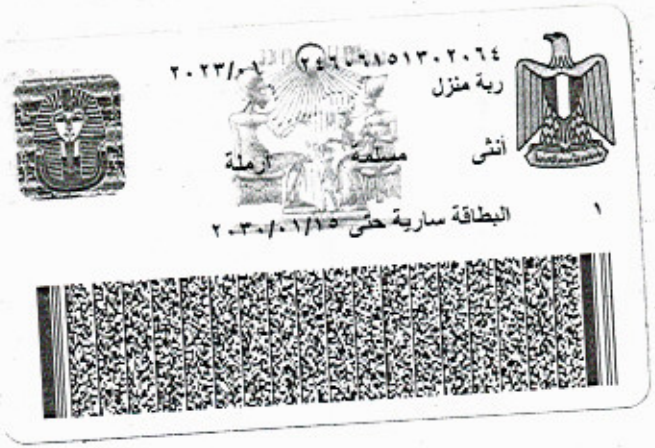
تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

GUSTAVE ROUSSY INTERNATIONAL COURSE

Patient Name	:الأستاذه / ام السيد السيد محمد يوسف	Request No.	: 13002353
Patient No.	: 24606151302064	Request Date	: 28/09/2025
Path No.	: 2949/25	Verification Date	: 08/10/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 79 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

CLINICAL DATA
Right pathologially proved malignant breast mass

NATURE OF SPECIMEN
Tru cut biopsy, collected on 28/9/2025

GROSS EXAMINATION
Multiple grayish white tissue cores ranging in size from 0.3 cm up to 1 cm, totally submitted.
IHC technique: Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

MICROSCOPIC EXAMINATION
* Histological type: Invasive duct carcinoma with focal necrosis and myxoid change .
* Grade (differentiation +nuclear pleomorphism+ mitosis): (3+2+2) Grade 2
* DCIS: Focal , less than 1 %, cribriform, low grade .
* Angioinvasion: Absent
*Perineural invasion: Absent
• IPT (Positive control showed appropriate reactivity) :
* ER: Strong nuclear reaction in 95 % tumor cells (Allred score 8/8)
* PR: Strong nuclear reaction in 95 %tumor cells (Allred score 8/8)
* HER2: Moderate complete membrane reaction in >10 % of tumor cells (score 2+)
*SISH : NOT AMPLIFIED .
* Ki 67: 7 %

DIAGNOSIS

Verified by :
Dr.Heba Hasan

CAP ACCREDITED
غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Name: ام السيد السيد محمد يوسف	
MR. : 2376	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient has metastatic hormone-receptor-positive breast cancer with documented progression to the lung and pleura. The disease was noted to be progressing while on letrozole (Femara), and the patient has been receiving a combination of palbociclib (Ibrance) and fulvestrant (Fasoldex) since July 2025.

Subsequent evaluation confirmed metastatic breast cancer involving the lung, pleura, and bone, with estrogen receptor (ER) score 8/8, progesterone receptor (PR) score 8/8, and HER2-negative status. The patient was treated with letrozole from September 2022 until July 2025, after which therapy was switched to fulvestrant 500 mg and palbociclib 125 mg for new clinical target sites. A bone scan was performed for fulvestrant monitoring, and a treatment delay was noted due to neutropenia, with a recommendation to consider dose adjustment at the next cycle.

A computed tomography of the chest, abdomen, and pelvis performed on 5 February 2026, compared with the study from May 2025, demonstrated stable disease while on fulvestrant plus palbociclib 125 mg, pending laboratory results.

The patient now reports a cutaneous reaction after the most recent dose of palbociclib, describing bluish discoloration of the hands and lips. An echocardiogram has been ordered to evaluate cardiac function prior to further fulvestrant administration, and palbociclib has been postponed for one week. Repeat complete blood count shows hemoglobin 8.3 g/dL, total leukocyte count 1.81 ×10⁹/L with neutrophils 0.54 ×10⁹/L; a reduction of palbociclib to 100 mg is being considered for the next administration. The current plan is to obtain the echocardiogram, reassess blood counts, and resume therapy with the adjusted palbociclib dose.

تحريرا في ,

التاريخ :	19-09-2026	**الوقت :**	

الطبيب المعالج : علياء احمد أبو النصر شهاب الدين

يعتمد ,

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	: الأستاذه / ام السيد السيد محمد يوسف	Request No.	: 13002353
Patient No.	: 24606151302064	Request Date	: 28/09/2025
Path No.	: 2949/25	Verification Date	: 08/10/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 79 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Right breast mass, Tru cut needle (TCNB) biopsy:
Invasive duct carcinoma, Grade 2 with focal duct carcinoma insitu , less than 1 %
IPT:
* ER: Positive (score 8/8).
* PR: Positivee (score 8/8).
* HER2:Equivocal (score 2+).
*SISH : Not amplified .
*Average her2 copy No .3, her2/cep17 ratio:1.2
* Ki67: 7 %
* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr. Walaa Ibrahim Ghoneim
Dr. Heba El-shafeey

Verified by :
Dr.Heba Hasan

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Name : OM EL SAYED EL SAYED MOHAMED	Date : 05-02-2026 02:36
Branch : Portable	Medical No. : GRI-16376
Ref.Physician Prof : CT CAP +C	Ins.Company :

MULTISLICE POST CONTRAST CT EXAMINATION OF THE CHEST ABDOMEN AND PELVIS REVEALED: -

As compared to previous study dated 22/5/2025 (only report available), the current study reveled

I CHEST:

- Still seen right pathologically enlargedaxillary LNS, the largest is measuring 3x1.6cm
- Still seen bilateral breasts /cleavage dermal& subdermal nodules, the largest is 1.3cm on the left side
- Still seen bilateral multiple pulmonary nodules associated with thick interlobular septations& interstitial coarsening, the largest is seen at the right upper lobe (10mm)
- Still seen right pleural thickening with mild right pleural effusion
- Still seen right lower lobe calcified nodule
- Still seen retrocaval& subcarinal calcified LNs
- The previously mentioned left localized pleural effusion is not currently seen, only pleural thickening is noted
- Normal CT appearance of the heart and great vessels.

II ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Calcular GB
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

III EXAMINED SKELETON:

Still seen multiple sclerotic lesions involving the axial& appendicular skeleton

OPINION:

- Overall stationary course.

Best regards

Dr. NERMEEN RAMADAN,MD

CT Scan
[[DOCTYPE:CT_SCAN]]

Name : OM EL SAYED EL SAYED MOHAMED
Date : 05-02-2026 02:36
Branch : Portable
Medical No. : GRP16376
Ref Physician Prof. : CT CAP +C
Ins.Company :

MULTISLICE POST CONTRAST CT EXAMINATION OF THE CHEST ABDOMEN AND PELVIS REVEALED:-

Findings:

- Multiple bilateral small parenchymatous and subpleural pulmonary nodules of average 5 mm; widely scattered within both lung fields, .
- Diffuse right pulmonary interstitial septal thickening is seen involving right lung associated with peribronchial thickening; likely suggestive of lymphangitis carcinomatosis.
- Mild right pleural effusion is noted.
- No C.T evidence of enlarged hilar or mediastinal lymph nodes.
- No pleural or pericardial effusion.
- Normal CT appearance of the heart and great vessels.

I- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.

CT Scan
[[DOCTYPE:CT_SCAN]]

Name : OM EL SAYED EL SAYED MOHAMED
Date : 05-02-2026 02:36
Branch : Portable
Medical No. : GRI-16376
Ref Physician Prof : CT CAP +C
Ins Company :

- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Multiple lytic and sclerotic lesion seen in the sternum, ribs, vertebral

OPINION:

- **Multiple bilateral pulmonary metastatic nodules and right pulmonary lymphangitis carcinomatosis**
- **Multiple bilateral pulmonary nodules with evidence of lymphangitis carcinomatosis; likely metastatic.**

Dr. Asser Ahmed

Dr. Asser Ahmed

CT Scan
[[DOCTYPE:CT_SCAN]]

Medical No. : 9110
Patient's Name: om elsaied elsaied mohamed
Date: 2026-09-13
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISLICE CT EXAMINATIUON OF THE CHEST ,ABDOMEN AND PELVIS WITH CONTRAST ADMINISTRATION REVEALED:-

- No appreciable changes as regards to the bilateral well-defined densities scattering in the breasts.
- Pathologically-enlarged right axillary lymph nodes are seen with loss of their fat contents and the largest one measures about 31x20mm(versus 30x16mm in the previously study).
- Reduced right lung capacity with ill-defined pulmonary nodules still seen scattering in both lung fields,with increasing size and the largest one seen in the anterior segment of left upper pulmonary lobe and measures about 19mm.
- Bilateral thickened interstitium with thickened interlobular septae scattering in both lungs,more on the right side.
- No patchy areas of consolidations or foci of calcifications noted.
- Retro-caval and subcarinal lymph nodes are still seen ,and globally measure about 40x18mm in the carinal region.
- No other sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- Patent central airways.
- Mild right pleural effusion and bilateral minimal pleural thickening.
- Minimal peicardial effusion.
- Average size of the heart with atherosclerotic changes of its big vessels.
- Preservation of the osteolytic /osteosclerotic bony lesions scattering in the skeleton with involvement of more bones as well.
- Average sized liver,measuring about 15 cm, showing a homogenous parenchymal attenuation and enhancement pattern.
- No sizable abnormally-enhancing focal lesions or dilated biliary radicles.
- Patent PV.
- Average sized spleen,measuring about 7.4 cm. No focal lesions seen inside.
- Gall bladder stones are seen,globally measuring about 26x18 mm with its CT density of 714 HUs.
- Thickened both adrenal glands,more on the left side.
- Normal CT appearance of the pancreas, functioning both kidneys(apart from a tiny left upper pole cyst,measuring about 7mm), aorta and IVC.

Medical No. : 9110
Patient's Name: om elsaied elsaied mohamed
Date: 2026-09-13
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

- Newly-developed aorto-caval,pre- and left para-aortic lymph nodes are seen ,globally measuring about 37x16 mm in dimensions at the former group.
- No other sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- No intra-vesical stones,solid masses,wall thickening or diverticulosis seen.
- The rest of the pelvic structures are unremarkable.

IMPRESSIONS:

- **COMPARE TO PREVIOUS STUDY DATED 5/2/2026**
- **No appreciable changes as regards to the bilateral well-defined densities scattering at different locations in the breasts,denoting a stable course.**
- **Pathologically-enlarged right axillary(with increasing size), retro-caval,subcarinal lymph nodes and newly-developed aorto-caval,pre-and left para-aortic lymph nodes,with ill-defined pulmonary nodules still seen scattering in both lung fields,with increasing size, increasing the extension of the osteolytic /osteosclerotic bony lesions scattering in the skeleton and thickened both adrenal glands,more on the left side,likely to be progressive metastatic lesions.**
- **Bilateral thickened interstitium with thickened interlobular septae scattering in both lungs,more on the right side,likely to represent a lymphangitis carcinomatosis.**
- **Calcular cholecystitis.**

Prof/Hanan El-Ahwal.

Dr.Hanan Elahwel

جوستاف روسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

البنك المصرية هي مصر

10/12/2026

MDT FORM

Name:	OM El-Sayed El-Sayed Mohamed Yousef	**MRN:**	2376
Age:	80 Y	**ID:**	24606151302064
Menopausal Sta		**Phone:***	01124577229
NRS SCORE:		**Marital Status:**	
BMI:		**Family History:**	
Gender			
CO Morbidity:		**Reason Of MDT:**	MBC
Surgical History:*	nad FTN	**ECOG Performance Status**	○ 0 ○ 1 ◉ 2 ○ 3 ○ 4

Clinical Data:

BX:

ER: **PR:** **HER2:** **KI67:**

History:

POSTM MBC TO LUNG ,PLEURA ,BONE ER8/8 PR8/8 HER2-VE REC FEMARA FROM 9/2022 TILL 7 /2025 DP ST FASOLDEX ,IBRANCE FOR NEW CTS ,BS FOR FASOLDEX 500 IBRANCE 125 10 months last CT CAP --> DP

Sonomammography:

Biomarker/Molecular

Mets Workup:

CT CAP 13-9--2026 --> DP of bone and adrenal mass MRI Brain --> not mets PS 2

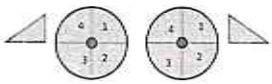
Recommendations:

for Afintor + Aromasine

MDT Outcome & Plan

For 3rd line ET; aromazine/affinitor (start at 5mg for 2 weeks then consider increase according to tolerability & BI glucose level).

R L



T N M

	Right Side				Left Side			
	Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د/شيماء لاشين	د/امنية عيسوي		

12/10/2026

Omnia G.
12/10/2026

[[DOCTYPE:MRI]]
M.R.I

Medical No. : 9110
Patient's Name: om elsaied elsaied mohamed
Date: 2026-09-30
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MRI OF THE BRAIN

Technique :

Axial T1, T2 ,FLAIR,DWI & ADC
Coronal T2
Sagittal T1
Postcontrast T1

Findings:

- Multiple bilateral cerebral periventricular foci of abnormal MR signal are seen displaying low signal on T1 that turned bright on T2 & FLAIR with no evidence of contrast uptake. No mass effect or perifocal oedema.
- Mild dilatation of the supra and infra tentorial ventricular system. Bright T2 & FLAIR signal are seen framing the periventricular white matter.
- No midline shift or deformity
- Prominent cortical sulci and extra axial CSF spaces.
- No extra axial collections or intra axial masses.
- No cerebello pontine angle masses.
- No gross posterior fossa abnormalities.
- Partial empty sella.
- Few bilateral calvcarial áreas of altered MR marrow signal eliciting low T1 and T2 WI signal with faint contrast uptake.

OPINION:

Multiple bilateral cerebral old lacunar infarctions.
Diffuse subcortical arteriolo-sclerotic leukoencephalopathy.
Involutional brain changes.
Partial empty sella.
Few bilateral calvcarial áreas of altered MR marrow signal for bone scan.

Dr.sherif Wagdy

[[DOCTYPE:PATHOLOGY_REPORT]]
Pathology Report

GUSTAVE ROUSSY INTERNATIONAL Egypte

Patient Name	: الأستاذه / ام السيد السيد محمد يوسف	Request No.	: 13002353
Patient No.	: 24606151302064	Request Date	: 28/09/2025
Path No.	: 2949/25	Verification Date	: 08/10/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 79 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Right breast mass, Tru cut needle (TCNB) biopsy:
Invasive duct carcinoma, Grade 2 with focal duct carcinoma insitu , less than 1 %
IPT:
* ER: Positive (score 8/8).
* PR: Positivee (score 8/8).
* HER2:Equivocal (score 2+).
*SISH : Not amplified .
*Average her2 copy No .3, her2/cep17 ratio:1.2
* Ki67: 7 %
* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr. Walaa Ibrahim Ghoneim
Dr. Heba El-shafeey

Verified by :
Dr.Heba Hasan

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

[[DOCTYPE:BONE_SCAN]]
Bone Scan

ELSAYED MOHAMED, OM Patient ID: 122019547 DOB: 15-Jun-1946 Date: 29-Jul-2025

Patient Lastname: ELSAYED MOHAMED
Patient Firstname: OM ELSAYED
HIS Patient ID: 122019547
RIS Patient ID: 310235
Date of Birth: 15/06/1946

Procedure: 70030, Bone Scan
Medical Question:
Clinical Comment:

========== REPORT TEXT ==========

99mTc MDP Bone Scan Report (29/07/2025)
* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of 99mTc MDP
Revealed:

Unfortunately, previous bone scan images are not available for comparison.

" Non-uniform tracer uptake is seen involving most of the spine and parieto-occipital skull bones? Remitted lesions?? otherwise for correlation with clinical and radiological data
" Arthritic changes are noted over both shoulder and both knee joints.
" Both kidneys are visualized.
Conclusion:
" Features of partially remitted osseous lesions as described for correlation with clinical and radiological data
" Follow up is advised.

Much Obliged
Mahmoud Akram
Huda Fathy,MD

========== REPORT TEXT ENDE ==========

[[DOCTYPE:CT_SCAN]]
CT Scan

Medical No. : 9110
Patient's Name: om elsaied elsaied mohamed
Date: 2026-09-30
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISLICE CT EXAMINATION OF THE CHEST WITHOUT CONTRAST REVEALED:-

- Bilateral scattered breast densities with right axillary lymph node measuring 20 x 18 mm seen for sono-mammography.
- Multiple bilateral pulmonary nodules, some of them being calcified, largest measuring 8.7 mm at the right lower lobe.
- Bilateral thickened interstitium with thickened interlobular septae scattering in both lungs,more on the right side.
- Minimal right pleural effusion.
- Minimal right pleural thickening.
- Retrocaval lymph nodes largest measurnig 19 x 13 mm.
- No left pleural or pericardial effusion.
- Normal CT appearance of the heart and great vessels.
- Intact bony thoracic cage.
- Bilateral elastofibroma dorsi.
- Upper abdominal cuts showed gall bladder stones.

Opinion:-

- **Multiple bilateral pulmonary nodules.**
- **Bilateral thickened interstitial and interlobular septal thickening.**
- **Minimal right pleural effusion and pleural thickening.**
- **Retrocaval enlarged lymph node.**
- **For PET CT assessment.**

Dr.sherif Wagdy